

EBOLA VIRUS DISEASE

Democratic Republic of the Congo

External Situation Report 01 Revised Version

Date of issue: 15 September 2025

Data as reported by: 14 September 2025

Situation update	¹ Cases	Deaths	CFR
	 36	 16	44.4%

On 4 September 2025, the Ministry of Health of the Democratic Republic of the Congo (DRC) declared an outbreak of Ebola Virus Disease in Bulape Health Zone, Kasai Province, following laboratory confirmation by real-time polymerase chain reaction (RT-PCR) and GeneXpert assays at the National Institute of Biomedical Research (INRB) in the capital Kinshasa. This is the 16th outbreak of Ebola virus disease in the Democratic Republic of the Congo.

The presumptive index case, a 34-year-old pregnant woman with 34 weeks of gestation from Tshitekeshi, Bulape Health Area, Bulape Health Zone, reportedly developed symptoms on 10 August 2025 and was admitted in labour to the Obstetrics and Gynaecology ward at a local hospital on 20 August 2025, with high-grade fever and acute onset of bloody watery diarrhoea. She died on 26 August 2025 while on admission and was subsequently buried in the community without adherence to safe and dignified burial practices. No sample was collected prior to burial. The source of infection of the index case is still under investigation.

On 22 August 2025, a nurse who provided care to the presumptive index case developed similar signs and symptoms and later died on 1 September 2025. Two other health workers, a laboratory technician and another nurse from the same hospital, also experienced onset of signs and symptoms on 24 and 28 August 2025 respectively and subsequently died.

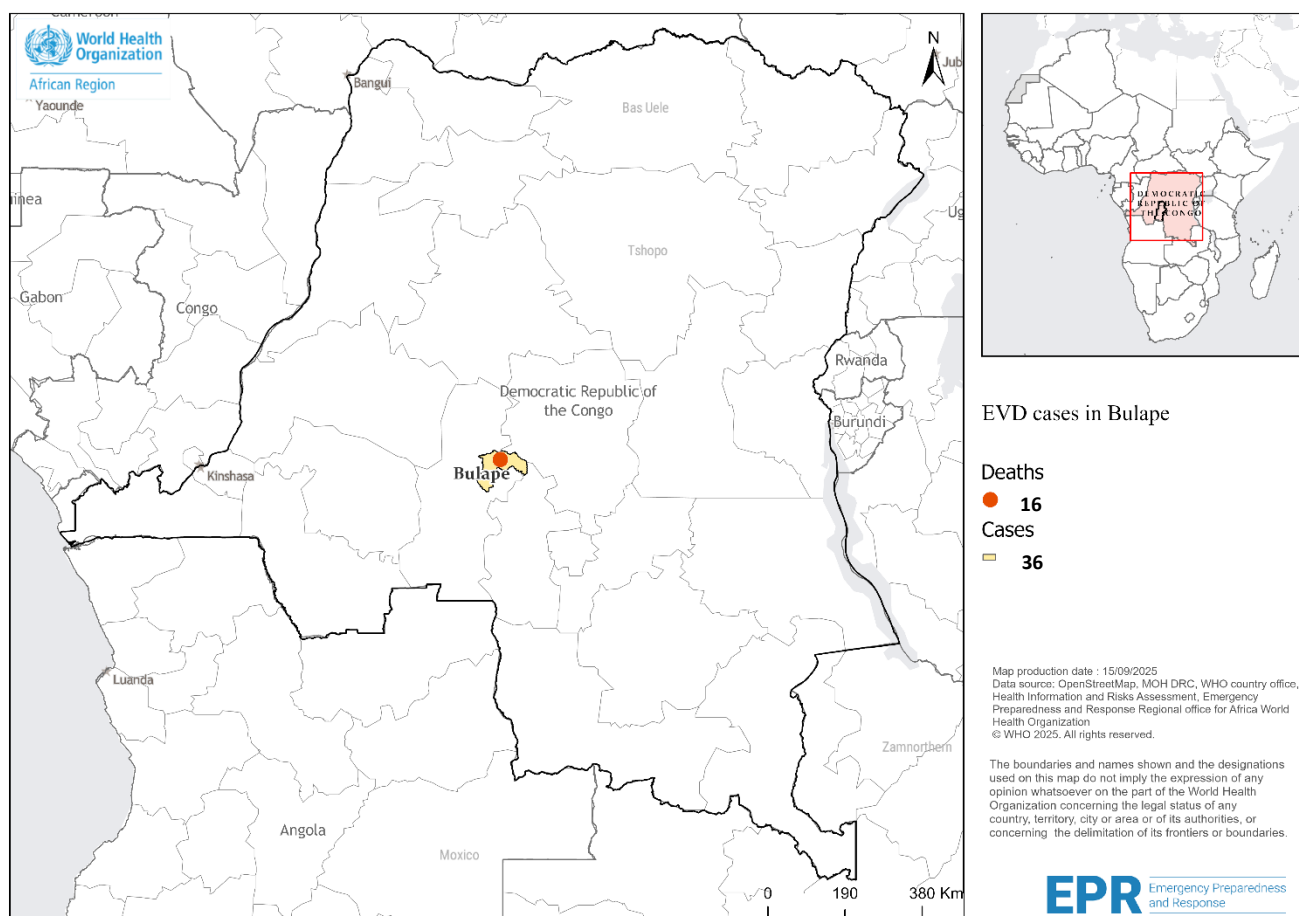
The cases and deaths among health workers who provided care to the index case, subsequent cases among close relatives and family members, coupled with their clinical presentations raised suspicions of a viral haemorrhagic fever outbreak, leading to alerting of the national health authorities on 1 September 2025. Subsequently, of the six samples collected from suspected cases, five tested positive for Ebola virus disease on 3 September 2025, thus confirming the outbreak in Bulape Health Zone.

As of 14 September 2025, a total of 36 laboratory confirmed cases, including 16 deaths (case fatality ratio 44.4%) have been reported from Bulape Health Zone, Kasai Province, Democratic Republic of the Congo¹. Four of the cases are among health workers. A total of 885 contacts have been listed and under follow-up, with 715 (80.8.0%) seen on 14 September 2025.

¹ The total case count of 54 cases, including 29 deaths, reported in the initial version of External Situation Report 01 has been revised to 36 confirmed cases, including 16 deaths. This revision reflects only confirmed cases at this stage, excluding probable and suspected cases. As investigations continue around several suspected and probable cases, these numbers will be updated in the next situation report.

Genomic analysis showed a 99.5% similarity to the genome of the 1976 Yambuku-Mayinga outbreak, suggesting that the current outbreak represents a new zoonotic spillover event and is not directly linked to the previous outbreaks in the province.

Figure 1: Geographical distribution of Ebola virus disease cases and deaths, Kasai province, Democratic Republic of the Congo, 14 September 2025



2. Public Health Response

Coordination

- The Ministry of Health, through the National Public Health Institute (INSP) and the National Public Health Emergency Operations Centre (COUSP), is coordinating the overall response to the outbreak, with technical support from WHO and health partners. A 3W (who, what, where) database is being updated to map partners involve in the response and support coordination efforts.

- Following the initial deployment of a national multidisciplinary rapid response team, supported by WHO, Médecins Sans Frontières (MSF), UNICEF and others, the Ministry of Health has now established and deployed a national Incident Management Team in Bulape Health Zone to enhance response activities and coordination across the different technical areas. Daily coordination meetings are being held to plan and guide response activities.
- WHO has also deployed 48 national and international experts, with 31 stationed in Bulape Health Zone to support the government response efforts. Other health partners supporting response efforts in Bulape include Africa CDC, International Organization for Migration (IOM), UNICEF, MSF, the Alliance for International Medical Action (ALIMA), the Congolese Red Cross, and US Centers for Disease Control and Prevention. Others offering support include the International Medical Corps (IMC), Family Health International 360 (FHI360), Program for Appropriate Technology in Health (PATH), Village Reach, World Food Program (WFP), UNFPA, the GAVI Alliance, and World Bank.

Surveillance and Laboratory

- Surveillance has been intensified in Bulape Health Zone and proximal areas. An alert system for reporting signals and suspected cases is being put in place, and all reported cases are being investigated.
- Contact tracing activities to list and follow-up all contacts of cases have started. As of 14 September 2025, a total of 885 contacts have been identified and are under follow-up for a period of 21 days from their respective date of last contact with a case.
- Active case finding activities are being implemented, including screening of passengers leaving and entering the affected areas. Mortality surveillance for systematic reporting and testing to rule out Ebola in all deaths has commenced.
- On 3 September 2025, INBR deployed a mobile laboratory in the Bulape Health Zone to facilitate rapid turn-around of the testing of samples. Samples are also being sent to INBR in Kinshasa for quality control and reference testing. As of 14 September 2025, samples from 87 suspected cases have been tested, with 36 positive for Ebola virus disease and 51 negative.
- Genomic sequencing of samples is also being performed at rapid speed. Sequencing results of the initial five positive samples were shared publicly less than 24 hours after confirmation of the outbreak. The results showed a nucleotide similarity of 99.5% to the Ebola virus genome of the 1976 Yambuku-Mayinga outbreak, suggesting a new zoonotic spillover event that is not directly linked to previous outbreaks of 2007 and 2008/2009 in the Kasai province.
- WHO and partners are working to ensure laboratory supplies are available and testing continues at the mobile laboratory in Bulape Health Zone. There are plans to establish additional mobile laboratories to ensure rapid turnaround time.
- Two GeneXpert and one Biofire machines have already been deployed to Bulape Health Zone. Additionally, supplies are pending in the coming days.

Case Management

- An Ebola Treatment Centre (ETC) with 18 beds has been set up at the Bulape General Hospital with the support of WHO and health partners. As of 14 September 2025, a total of 16 cases (12 confirmed and 4 suspected) were admitted. Bed occupancy rate is 88.9%.
- To date, 14 patients have been administered treatment with the monoclonal antibody (mAb114). A treatment course sufficient for approximately 20-25 patients is currently available at the Bulape ETC.
- Furthermore, supportive and nutritional care are being provided, though operational challenges remain significant.
- A surge team of health care workers has been deployed by the MoH and Institut National de Santé Publique (INSP) to support clinical care, including trained specialists from the pool of rapid responders and the national emergency medical team.
- ALIMA, MSF, and INSP are actively supporting case management activities.
- Additional staff are being deployed to relieve existing staff at Bulape General Hospital who were listed as contacts so as to ensure continuity of essential services.

Vaccination

- The International Coordination Group (ICG) has approved 43,840 doses of rVSVΔG-ZEBOV-GP (Everbo) Ebola vaccine for a reactive vaccination campaign in Kasai Province. This is in addition to the 2 000 doses that was initially approved and arrived in Kinshasa on 12 September 2025.
- On 12 September 2025, a total of 360 doses of Ebola vaccine were delivered to Bulape Health Zone and vaccination started on 13 September 2025. A ring vaccination approach targeting contacts and potential-contacts, healthcare and frontline workers (HCW/FLW) at high risk is being implemented. This will be complemented by a targeted geographic approach in all hotspots where cases have been confirmed when more vaccine doses arrive.
- As of 14 September 2025, a total of 133 HCW/FLWs at the Bulape General Hospital and 150 contacts have been vaccinated. Forty-five healthcare workers have been trained to constitute teams for vaccination.
- Installation of the Ultra Cold Chain equipment is ongoing in Tshikapa, Kananga and Mweka to accommodate a bigger volumes of vaccine doses.

Infection prevention and control (IPC)

- An initial assessment of compliance to IPC standards was conducted at the Bulape General Hospital and Bulape Health Centre, with results showing very low compliance. Key gaps in screening, isolation

measures, including the use of personal protective equipment (PPE) , environmental hygiene and waste management were found. Immediate remedial measures are being implemented, including the setup of screening at the entry to the Bulape General Hospital.

- To reinforce IPC measures, 77 health workers in Bulape Health Zone have been trained on basic IPC practices, screening and triaging of patients using case definitions, standard and transmission-based precautions including safe use of PPE and proper waste management.
- Additionally, IPC measures such as the conduct of safe and dignified burials, disinfection of households of confirmed cases, and improved handling and disposal of medical waste at the Bulape ETC have been instituted.
- Three Safe and dignified burial teams have been activated and are operational in three priority areas of the Bulape Health Zone. As of 14 September 2025, a total of 13 safe and dignified burials have been supervised by the burial teams.

Prevention for Sexual Exploitation, Abuse and Harassment

- Engagement with Provincial Health Authorities: On 14 September 2025, a meeting was held with the Provincial Minister of Health of Kasai Province to discuss mainstreaming prevention of sexual exploitation, abuse, and harassment (PRSEAH) in the response to the outbreak.
- PRSEAH focal points have been identified among partner organizations and government personnel deployed to support response activities in Bulape, Kasai Province. All WHO deployed staff have received training on PRSEAH and re-signed the code of conduct.

Risk communication and community engagement (RCCE)

- Risk communication and community engagement activities are being scaled up in Bulape Health Zone and proximal areas through household visits, community briefings, and sensitization sessions. Between 10 and 11 September, over 300 people were directly reached through household visits.
- Community leaders and community health workers have been engaged to enhance awareness on Ebola virus disease signs and symptoms, transmission, and preventive measures.
- Leaflets have been distributed, mapping of churches and local leaders completed, and targeted interventions carried out to address refusals and rumours, including cases of resistance to isolation and transfer to treatment centres.
- RCCE activities are being mainstreamed into contact tracing and vaccination efforts in the affected communities.
- A rapid assessment was conducted and finalized on 9 September 2025, providing baseline information on knowledge, attitude, and practices in the community. The assessment found low community knowledge about the disease, defiance towards health system, and high-risk behaviours, among others. The results of this assessment is being used to tailor RCCE interventions.
- As of 13 September, three instances of refusal of admission to the ETC have been resolved.
- To continue to expand and intensify RCCE activities, community health volunteers (RECOs) in the affected health areas are being trained. So far, 65 RECOs have been trained to create awareness and engage community members.

Operations Support and Logistics

- WHO and partners have rapidly scaled-up operational and logistical support in synergy with the efforts of the government of the Democratic Republic of the Congo to respond to the outbreak.
- To fast track delivery of supplies and personnel, a collaboration between WHO, WFP and the United Nations Organization Stabilization Mission in the Democratic Republic of the Congo (MONUSCO) has established a temporary airbridge for two weeks using a helicopter from Kananga to Bulape and Mweka.
- Over 14 tons of essential medical supplies and equipment have been delivered to strengthen frontline response efforts with air support from the WFP and MONUSCO to access the affected health zone.
- A 40-foot container truck with essential supplies has been dispatched from nearby Tshikapa to Bulape while an additional 600 kilogram of medicines, vaccines, and personal protective equipment (PPE) is enroute from Kananga. Additional shipment of 2 tons of PPE, chlorine, and other supplies have been scheduled.
- An ETC was rapidly established at Bulape Hospital with 18 beds, medical supplies and consumables provided, and 4000 litres of water supplied. A supply chain system has been established and a new ETC with expanded capacity is under construction.

Situation Interpretation

The current Ebola virus disease outbreak in Bulape Health Zone, Kasai Province, represents a serious public health threat. The outbreak originated from a likely new zoonotic spillover and has already resulted in sustained human-to-human transmission, including infections and deaths among health workers, which heightens the risk of further nosocomial amplification. The crude case fatality ratio of 50.0% underscores the high impact on human health. Given Bulape's connectivity to larger population hubs such as Tshikapa and Kananga, and the ongoing movement of people across provincial and national borders, there is a real risk of further geographic spread.

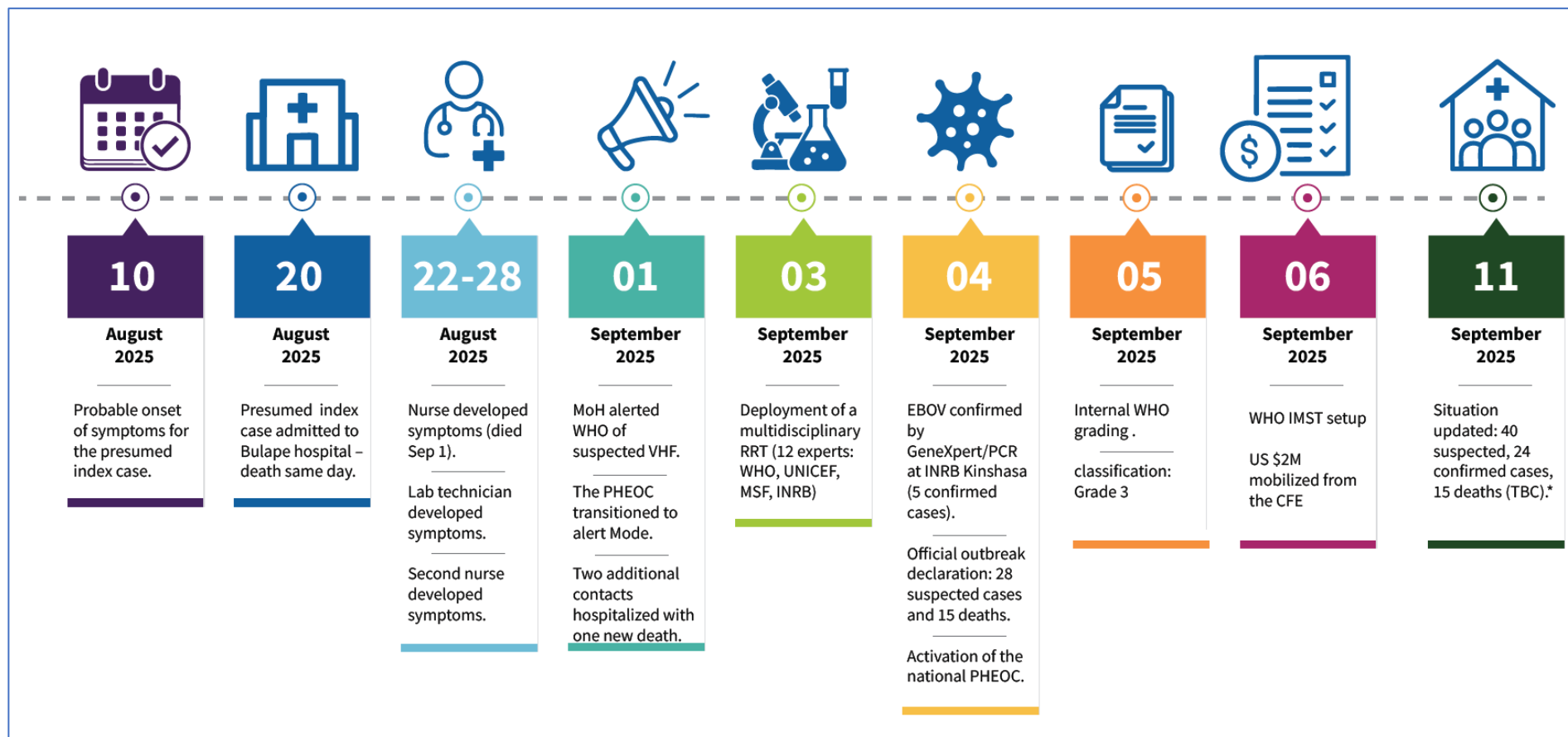
The rapid response mounted by national authorities and partners is crucial, with enhanced surveillance through contact tracing, improved clinical care including treatment with monoclonal antibody therapy, and the initiation of vaccination serving as critical tools to control the outbreak. Consolidating these gains by strengthening surveillance, reinforcing IPC standards, expanding vaccination, and sustaining community engagement will be critical to containing transmission in Bulape and reducing the risk of further spread.

Domestic and international traffic-related measures and cross-border health

No international traffic-related measures are currently warranted. Health authorities are reinforcing surveillance at border crossings, including health screening, proactive risk communication with travellers at points of entry/points of crossing (PoEs/PoCs), sensitization for PoE staff to detect, report and manage suspected cases, integration of border communities in affected areas into early warning systems and the national surveillance

network, as well as coordination with IOM. WHO continues to monitor cross-border risks and provide technical support to mitigate the risk of cross-border spread.

Timeline of key events



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Correspondence on this publication may be directed to:

Dr Patrick Otim RAMADAN
Program Area Manager
Epidemics and Humanitarian Health Response (EMR)
WHO Emergency Preparedness and Response
WHO Regional Office for Africa

P O Box. 06 Cité du Djoué, Brazzaville, Congo

Dr Etien Koua
Program Area Manager
Health Emergency Intelligence, Surveillance and Laboratory Programme
WHO Emergency Preparedness and Response
WHO Regional Office for Africa
P O Box. 06 Cité du Djoué, Brazzaville, Congo

Email: afrooutbreak@who.int / afrgoafroshoc@who.int

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